

PAWSIC QAPI Post-Test

Skin Health & Wound Management · 25-Question Knowledge Check

INSTRUCTIONS

Select the **single best answer** for each of the 25 questions by circling or marking the letter. This knowledge check is drawn from the Assessment-Action Blueprint (PAWSIC 2026). Completing it is part of earning your Skin Health & Wound Management PAWSIC QAPI Certificate.

Name: _____

Date: _____

Facility: _____

Role / Discipline: _____

Score: _____ / 25 Percentage: _____ % Pass / Fail: _____

Section 1 — Foundations & Wound Types (Questions 1–5)

1. According to the program, wounds are best understood as:
 - A. An unavoidable consequence of aging
 - B. A measurable indicator of center health and systems performance
 - C. Solely a nursing responsibility
 - D. A problem only relevant during survey
2. The presentation states that litigation in post-acute care is most often caused by:
 - A. Use of advanced therapies
 - B. Over-documentation of wounds
 - C. Making too many specialist referrals
 - D. Failure to properly communicate with the patient and family
3. Which chronic wound is identified as the most common leg ulcer in the U.S., making up 60–80% of all leg ulcers?
 - A. Venous leg ulcer (VLU)
 - B. Diabetic foot ulcer (DFU)
 - C. Arterial ulcer (AU)
 - D. Pressure injury (PI)
4. According to the program, Stage 1 and Stage 2 pressure injuries can develop within as little as:
 - A. 24 hours
 - B. 1 week
 - C. 1–2 hours
 - D. 30 days
5. Skin Changes at Life's End (SCALE), including Kennedy Terminal Ulcers, are:
 - A. Always coded as Stage 4 in Section M
 - B. Not primarily caused by pressure and not coded in Section M
 - C. Considered avoidable in every case
 - D. Never recognized by F686

Section 2 — Patient-Centered Care, Documentation & Process (Questions 6–12)

6. Best practice for skin checks includes all of the following EXCEPT:
 - A. Conducting facility-wide “skin sweeps”
 - B. Requesting permission and explaining the “why”
 - C. Performing them as part of standard daily care
 - D. Maintaining the resident’s privacy and dignity

7. The recommended frequency for skin/wound risk assessments is:
 - A. On admission only
 - B. Monthly only
 - C. Annually
 - D. Upon admission, weekly x4, quarterly, and change in condition
8. When building the care plan, the program advises that you should:
 - A. Copy the full daily wound assessment into the care plan
 - B. Leave wounds off the care plan to avoid duplication
 - C. Align the care plan with the assessments rather than repeating them
 - D. Update the care plan only at the quarterly review
9. Routine wound documentation should be completed upon admission/new wound, weekly, and:
 - A. Upon an unanticipated decline
 - B. Only at discharge
 - C. Every 90 days
 - D. Only when requested by survey
10. The Wound Log is best described as:
 - A. A replacement for the EMR
 - B. A Medicare billing document
 - C. The same record as the MDS
 - D. A single view of every wound and a window into systems and processes
11. Per the treatment protocol guidance, which element MUST be part of every treatment regimen?
 - A. Daily sharp debridement
 - B. Systemic antibiotics
 - C. Proper wound cleansing and pain management
 - D. Negative pressure wound therapy
12. The CMS F686 Critical Element Pathway (CEP) is used as a root-cause analysis primarily for:
 - A. In-house acquired pressure injuries (especially Stage 3, 4, DTI, Unstageable) and declining/infected PIs
 - B. Every resident, regardless of wounds
 - C. Present-on-admission wounds only
 - D. Surgical site infections only

Section 3 — Team Roles, QAPI & Action Planning (Questions 13–17)

13. Performing the new and weekly skin/wound assessment and risk assessment is primarily the role of the:
 - A. CNA
 - B. RN
 - C. LPN
 - D. Registered Dietitian
14. Which team member is primarily responsible for daily observation and reporting of skin issues?
 - A. RN
 - B. Medical Director
 - C. Social Services
 - D. CNA
15. The Skin Health Team Lead is primarily responsible for:
 - A. Performing every dressing change personally
 - B. Billing Medicare Part B for dressings

- C. Oversight and support for the center's entire skin health and wound management program
 - D. Setting the facility operating budget
16. Which of the following is a properly written SMART goal?
- A. "I want to improve our team's communication and heel off-loading."
 - B. "Implement a weekly 15-minute stand-up starting Monday to improve staff adherence to heel off-loading to >85% within 30 days."
 - C. "We will improve wound care soon."
 - D. "Everyone should off-load heels better."
17. As a program "litmus test" of success, newly in-house acquired pressure injuries should ideally be:
- A. Found at Stage 1 or 2
 - B. Found at Stage 3 or 4
 - C. Left off the wound log
 - D. Reported only at the quarterly review

Section 4 — Infection Control & Enhanced Barrier Precautions (F880) (Questions 18–25)

18. The presentation describes the relationship between F880 (infection control) and F686 (wound care) as:
- A. Unrelated regulations
 - B. Mutually exclusive
 - C. Relevant only during a survey
 - D. Inseparable — infection-control requirements govern every dressing change
19. Enhanced Barrier Precautions (EBP) apply to:
- A. Only residents with a confirmed MDRO
 - B. Only residents already on Contact Precautions
 - C. All residents with a chronic wound or an indwelling device, regardless of MDRO status
 - D. Only new admissions
20. Which of the following triggers Enhanced Barrier Precautions?
- A. A PICC line
 - B. A peripheral IV
 - C. A one-time blood draw
 - D. A fully healed surgical incision
21. The correct donning (putting on) order for EBP personal protective equipment is:
- A. Gloves → Gown → Hand hygiene
 - B. Hand hygiene → Gown → Gloves
 - C. Gown → Gloves → Hand hygiene
 - D. Hand hygiene → Gloves → Gown
22. A chronic wound whose drainage is contained by an intact dressing calls for:
- A. Contact Precautions with room restriction
 - B. No precautions of any kind
 - C. Enhanced Barrier Precautions (resident is not room-restricted)
 - D. Droplet precautions
23. Under Enhanced Barrier Precautions, gown and gloves are required:
- A. Every time any staff member enters the room
 - B. Only during meal service
 - C. Only when an MDRO is confirmed
 - D. For high-contact wound care activities — not simply to enter the room

24. Per aseptic technique (ANTT) at the bedside, clean gloves:
- A. Must never contact the wound bed
 - B. May touch the wound bed if changed afterward
 - C. Are optional for chronic wounds
 - D. Replace the need for hand hygiene
25. Which bedside finding is the SPREADING / EMERGENCY presentation?
- A. Periwound erythema with purulent drainage
 - B. Cellulitis extending beyond the wound margins, streaking, or crepitus
 - C. Fever
 - D. A small amount of serous drainage

Facilitator Answer Key & Scoring Guide

For scoring only — remove or withhold this page before distributing the test. Each item is worth 1 point (25 total). “Source” refers to the slide in the Assessment-Action Blueprint.

#	Ans	Source	Key point
1	B	Slide 3	Wounds reflect the interaction of chronic disease and care delivery and are a measurable indicator of center health and systems performance.
2	D	Slide 2	Litigation cause is the same as deficiencies, but most often due to failure to properly communicate with the patient/family.
3	A	Slide 7	VLU is the most common leg ulcer in the U.S. (60–80% of all leg ulcers).
4	C	Slide 19	Pressure injuries occur; Stages 1 and 2 can develop within just 1–2 hours.
5	B	Slide 8	SCALE/KTU/skin failure are not primarily caused by pressure and are not coded in Section M; F686 and RAI now both recognize KTU.
6	A	Slide 15	Skin checks should be part of standard daily care with privacy and permission; the program says to avoid “skin sweeps.”
7	D	Slide 15	Risk assessment frequency: upon admission, weekly x4, quarterly, and change in condition.
8	C	Slide 16	Don’t repeat assessments—align with them. Each wound is on the care plan with an aggregate of risks; daily-changing detail is not added.
9	A	Slide 43	Frequency: upon admit/new wound, weekly, and unanticipated decline (if PI, stage is required).
10	D	Slide 45	The Wound Log is NOT the EMR; it is a single view of every wound—a window into your systems and processes.
11	C	Slide 46	Proper wound cleansing and pain management must be part of the treatment regimen; treatments are per order and per MI.
12	A	Slides 41 & 69	Use the CEP for all IHA PIs (especially 3, 4, DTI, UN) and declining/infected PIs as an individual root-cause analysis.
13	B	Slide 25	RN: new and weekly skin/wound assessment and risk assessment.
14	D	Slide 25	CNA: daily observation and reporting of skin issues, and providing interventions.
15	C	Slide 31	The SHTL provides oversight and support for skin health and wound management for the center and ensures communication/continuity of care.
16	B	Slide 79	A SMART goal is Specific, Measurable, Achievable, Relevant, and Time-bound—only the option with a concrete metric (>85%) and a deadline (within 30 days) meets all five.
17	A	Slides 55 & 64	Litmus test: IHA are found at Stage 1 or 2, and Stage 1/2 heal within ~1 week to 30 days; CEP completed for all IHA PIs.
18	D	Slide 56	F880 and F686 are inseparable; infection-control requirements govern every dressing change. F880 is the most-cited deficiency in U.S. nursing homes.
19	C	Slide 57	EBP applies to ALL residents with a chronic wound OR an indwelling device, regardless of MDRO status (surveyed under F880 since April 1, 2024).
20	A	Slide 57	A peripheral IV does NOT trigger EBP—a PICC line does. Indwelling devices (central lines, catheters, feeding tubes, trachs) trigger EBP.
21	B	Slide 57	Donning order: Hand hygiene → Gown → Gloves. (Doffing reverses: Gloves → Gown → Hand hygiene.)

#	Ans	Source	Key point
22	C	Slide 58	The operative trigger is whether drainage can be contained. Contained by an intact dressing = EBP; uncontrollable drainage = Contact Precautions.
23	D	Slide 57	EBP PPE is for high-contact care (such as wound care)—not simply to enter the room.
24	A	Slide 59	Clean gloves must never contact the wound bed; identify and protect all “key parts.” Contaminated contact is a citable F880 finding.
25	B	Slide 60	Spreading/emergency: cellulitis beyond margins, streaking, crepitus—call 911; necrotizing fasciitis is a surgical emergency. (Local = notify same shift; Systemic = notify immediately.)

Answer distribution: A = 7 · B = 6 · C = 6 · D = 6 (of 25)